

Raven

The bell rang several times.

Caroline, already thinking about it before this new ringing, was beginning to feel like some sort of servant. She went to the window, meaning to say, “What do you want now?” and “I just fed you!” Instead she said, “What do you just fed want now!”

A caw, a laugh.

The local raven’s bell tolls were becoming more frequent. And Caroline wasn’t quite sure what to do about it, apart from reflecting on her mother’s warnings about exactly this. “If you give a raven a bell, she’ll surely ask for more,” she’d said. To which Caroline clarified that that was the point. The tinting on the old window had started to wear out—which was confusing in its own right since wasn’t tint applied from the inside—when Caroline finally said enough was enough. She needed her security deposit back, especially after that last hospital visit.

The psychiatrist had told her bipolar I with psychotic features. Or rather, the third one had repeated the diagnosis of the previous two. Caroline for her part hid her eyes when the social worker and not the doctor had told her the diagnosis. The social worker felt insulted too, which satisfied Caroline to a degree. She had no agency over the situation or diagnosis, so having some effect gave her some solace. Before that moment, the social worker and Caroline had built some rapport. On one occasion, the worker had asked Caroline about her perspectives on things, whether she thought aliens were real, and if Caroline could see a bright future ahead for the social worker. Caroline, albeit amused at the lack of professionalism, had done her best to add something of worth to the worker’s wonderings, but knew that her words were hollow at that point. She could feel in her answers that she wasn’t in tune anymore. Yet the worker was listening with furrowed brow and pen in hand.

Thinking on it now, Caroline began to wonder if “pen in hand” meant the social worker was waiting to jot down notes on her future, or if the worker was listening to Caroline with “furrowed brow” to discern the presence of Caroline’s alleged grandiosity. Woozily, Caroline laughed at it all. And jumped a little at a reminder-caw from her oh-so-neighborly raven. Did the worker know? When worker and client engage in conversation like that, how does either know things are being accurately perceived when each side is playing their own game?

At any rate, to be given her diagnosis from someone she felt (at the time) to be a believer of sorts, and for whom she felt bad, jarred her. It was insulting for everyone. Except the psychiatrist, who was insulated from any fallout. Delegate the message to the social worker, the worker bee. The one with fears and hopes and dreams and odd beliefs about aliens, Caroline had found.

On diagnostic receipt, Caroline shut down. Covered her eyes with the beanie

she was wearing. Felt betrayed that the social worker had used the language on her and felt betrayed by her doctor for not giving the news herself. Caroline wasn't surprised in the traditional sense of the word. Rather, she had come to this hospital with the intention of getting a second opinion. And all that came of it, all collapsing into this one moment with her social worker, was a repetition.

For many, diagnostic receipt is a rush of relief. "Ah, that explains so much!" or "It's so nice to know what I have, *finally*." The process is protracted, it's true. Our dearest Caroline, though? *Much* different. For one thing, this wasn't her first rodeo: previous messengers, previous receipts. More than that, Caroline felt alien to those around her. Those around her might term it "depersonalization" but she knew that one already and knew better. No, she rather felt that alien feeling, and like an animal in a literal sense. She carried in her a whole suite of patterns, behaviors, and loops. They saw these things within her and cast judgment on them. The comparison that made the most sense to her was a group of tide poolers walking round each microcosm of a pool naming everything in sight before moving on to the next. "We call this bipolar I with psychosis in our milieu; you will take special pills to be like us and live in our culture now," the tide poolers would whisper to her, the blue-ringed octopus. She'd flare her rings and do her best to deliver her neurotoxins into their bloodstreams. They tended to wear special gloves, and but of course they had the phylum memorized and the diagnosis codes clearly, cleanly printed on the receipts. They could have simply handed her the receipt right that moment themselves but doing so would require taking one's glove off. Dirty work for a *philatelist*. That was a good term for them. What better than a misnomer?

This go around in the hospital was not so peachy as the previous ones had been because she was required to take a 2-month, long-acting injection. "They want to strip me of my toxins," she thought with a blue-ringed jeer. Her shrink didn't trust her and therefore refused to let her go on any but his own terms. Sadly, for her the terms were two: an injection in the arm, like a flu shot, and another in the butt—not like a flu shot at all, that one.

The social worker didn't hide her disappointment at Caroline's blowup. She asked Caroline to take the beanie off her head or at least take it off her eyes. Caroline refused. The worker felt annoyed at this and at being punished for doing her job. Yet she understood Caroline's point all the same. Wanted to process and digest the symbolism of the blindness—but couldn't. The symbol passed in one vein and out an artery. Caroline's gift, she found, was the symmetry of her refusal to remove the beanie with the worker's own inability to process the symbol of that moment. The worker hated it. And so avoided it. For years.

Of all those she had helped over her career, Caroline seemed to be the one she helped least, had reached the least.

The worker was not just in charge of occasionally giving diagnoses to patients. She also led education sessions for the patients, usually at least once a week. These typically revolved around compliance, but sometimes focused on cognitive

distortions, coping mechanisms, or even sleep hygiene.

It had been several years since she ended her social work career and launched a successful photography business. She specialized in portraiture and felt that her time in the field as a social worker helped her capture what she saw in people.

What did she see? Life, mostly. It seems that easy because it is. Small moments of humanity and tenderness, the look in someone's eyes that shows an urge to be greater than what they are. The urge to reach for something far away, and the urge to grow for it, even if it means losing part of yourself in the process. The plant in the worker's window had a few massive leaves, for example, and a few smaller ones had begun to die out. Parts of ourselves, the worker realized, had greater appetites than others. Or were better suited to such-and-such environment than other slabs of who we are. She wondered how it was that people could go for a walk without having to fight back tears. Every person a pocket of decisions and aspirations and—It all overwhelmed her. She left social work because of this. Had to. She had spent her time in school developing this insight into people, their surroundings, and their presentations; it ultimately became too corrosive for her in that sphere of work. She preferred a different kind of capture and, most of all, she enjoyed how consensual her version of her new profession was.

How was it that people could go for a walk without crying, she wondered. Was it like a leaf quickly pruned before childhood or a stem cut before the teenage years wound down? Elbowed out of the way by the more socially high-yield behaviors, “those ones that didn't involve crying in public,” she wondered bitterly this time.

What comforted her most now was the privilege to capture parts lost, to sort of reanimate them again after short or long dormancy for the client. It startled her not long into the venture when she realized her clients were experiencing joy. They were seeing themselves. And the worker was the instrument of that seeing. From then on, the worker always made sure to be present when a batch of pictures were ready for the client. It annoyed some of them, until they realized the kind of work our worker was doing for them.

While her memory of giving Caroline her diagnosis had stuck with her, it was never the symbolism she ruminated on. Rather, the symbolism tended to be hurriedly swept off its feet and under the rug and out the door down the block. The logic of this was the worker's shame. Caroline's lack of consent. Her facelessness.

The worker was assigned this particular day's group session because compliance was especially low. Patients weren't taking their medications and weren't attending the various therapies provided by the hospital. Although music therapy remained steady in attendance during these periodic droughts, no doubt because it allowed patients the opportunity to request songs from the internet (which they didn't have access to), the handful of days leading up to the social worker's session were almost empty in the art therapies, including music. This wasn't completely new or unheard of. It happened in cycles and frustrated the

worker. “Why is it that you can put the solution to someone’s problems in front of them and they somehow figure out a way to avoid it? And when they have nothing better to do, too!” She’d ask this question and similar ones of her parents frequently during these dry spells. Initially, they laughed and made comments about parenthood. Eventually they grew bored, began to shrug. Pointed to the old adages, “horses and water,” they’d say.

With a huff, the social worker walked into that group compliance session she’d been assigned and let some of her frustrations out.

A few patients were still settling, shuffling around a bit to get comfortable. Many of them in unwashed clothes or oversized paper scrubs provided by the hospital. She wondered why they didn’t request the laundry services to take care of their personal clothes. Then thought of how people tend to live in the “real” world and felt her mind quiet itself just enough for her to form a thought. One she was willing to speak into the compliance session:

—Afternoon, everyone!

The thought had given her subliminal pep, perhaps an equal and opposite reaction to squalors chosen and unchosen.

—This afternoon’s session is going to focus on medication! How is everyone doing today? Anything good for lunch?

She wanted to inject the idea of medication into the session but buffer or dilute it with the mundane. Skim over it without skipping a beat and prime the patients in attendance with what they were about to face. In themselves, maybe even in each other. She hoped at least a few of them would hold some of the others accountable. Maybe they can hear it better from their own?

—Chicken and rice, lady. As always. Did you not know that?

—How’s everyone feeling? I’ll keep this session as brief as I can, so we can just do an abbreviated check in before we start discussing medication.

—Chicken and rice type of day, said one patient.

—Rice and chicken type of week if we’re being honest, said Caroline.

A few murmurs of agreement.

—Yes well it seems a normal day for you all then. I guess I’ll start by saying

The worker paused and felt the sudden weight of...everything swoop in on her.

—Please. Just take your medications guys. Just, it would help us all a lot. And I mean it’d help you too of course, but please. Just take them. It’s not that—

Caroline cut in with a raised hand. It caught the worker off guard. She called on Caroline with a flick of the hand, without realizing it till she heard her a few words deep.

—Are girls socialized to wear dresses?

The patients looked left, looked right. Not fully understanding. The worker put her hand to her forehead, headache reverberating from the back of her head to the front, where her fingers were now pressed.

—Sorry, sorry! I'll see myself out of this nonsense!

Caroline walked out politely, but stifling laughter. The worker intuited that the laughter was genuine, but also part plumage to this one. She enjoyed her moments of public critique and wanted it to show. She wasn't sorry.

—You're always welcome, Caroline. No need to excuse yourself.

The worker knew this last wasn't good enough to reverse the undermining Caroline just performed on her session. The comment, the worker could tell, subtly destabilized the room from there. The patients started voicing the more worldly problems of the situation, rather than what would make them better. They spoke about the cafeteria food and petty interactions they had with each other and staff. Talked shit about one of the art therapists. The worker added one more feather to her "Photography sounds nice" hat that afternoon and chastised herself for thinking she was going to let the patients have it when she started it up in the first place. They always... had other ideas. As was their "gift."

For her part, Caroline tapped the glass and waved from the inside till the raven flew off. She opened the window, grasped the bell's strand of rope and clanged it around a bit for good measure. Then wondered if this is how the social workers feel: making noise, waving their arms desperately to ward off the ones who cause trouble in hopes of protecting the naïve from biting questions. *Are* girls socialized to wear dresses?

With the worker cocooned away in her darkroom and Caroline refilling the birdseed and checking the fraying bell rope, they each murmured, *Yes*.